National Cancer Control Programme (NCCP) 00734 · clinician draft

weekly carboplatin AUC 2 / paclitaxel → dose-dense AC

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Phase 1 · weekly carboplatin / paclitaxel

Active medicines: Carboplatin + Paclitaxel

Emetogenic risk: Moderate · carboplatin AUC <4

Medicine	Dose / route	When
Ondansetron	8 mg orally twice daily	Day 1 National Cancer Control Programme (NCCP) alternative to 16 mg once daily; review QT / hepatic risks
Dexamethasone	8 mg orally	Day 1; reconcile paclitaxel premedication Do not add to the paclitaxel steroid course automatically
Dexamethasone	8 mg orally daily	Days 2–3 reference antiemetic row Reconcile the delayed period with paclitaxel steroids; one final individualised steroid prescription
Metoclopramide	10 mg orally up to three times daily as required	Subsequent days: duration to be confirmed Review contraindications and renal function

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment.
- Day 8 and later: no routine dexamethasone unless the consultant specifies it for reaction/risk; chlorphenamine continues and any famotidine omission requires consultant/local-policy agreement.
- Reconcile antiemetic and taxane dexamethasone into one prescription; never add duplicate steroid courses.
- Four 21-day cycles: paclitaxel on Days 1, 8 and 15; carboplatin AUC 2 on Days 1, 8 and 15.
- This is a weekly combination visit, not paclitaxel alone.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00734 v3 · p6 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-20 · SACTCheck v0.84.13 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Phase 2 · dose-dense AC

Active medicines: Doxorubicin + Cyclophosphamide

Emetogenic risk: High · AC

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 No routine Days 2–4 dexamethasone in this National Cancer Control Programme (NCCP) AC antiemetic row
Olanzapine	5 mg orally	Days 1–4 Review sedation, falls, interactions and glucose
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications

Additional phase-specific support

- Four further cycles (cycles 5–8): doxorubicin and cyclophosphamide on Day 1 every 14 days. granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) support is required for every dose-dense AC cycle; the team prescribes its product and timing separately.
- G-CSF every dose-dense AC cycle is required. Product, start day and duration must be prescribed using the local supportive-care policy; do not administer simultaneously with cytotoxic chemotherapy.
- Confirm cumulative doxorubicin exposure, cardiac assessment and venous-access plan.
- Cyclophosphamide Day 1: NCCP advises 2–3 L oral fluid, subject to the individual plan. Confirm suitability and adjust for fluid restriction, renal or cardiac disease.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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